

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

March 19, 2020

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
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Chief, Investigations Unit (b)(6),(b)(7)(C),(b)(7)(F)
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FROM: CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

SUBJECT: Mortality Review – Report of Findings
Anthony Oluseye AKINYEMI, (b)(6),(b)(7)(C)

Executive Summary:

On December 22, 2019, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Anthony Oluseye AKINYEMI. Mr. AKINYEMI, a 56-year-old Nigerian male, was in ICE custody from December 20 - 21, 2019, and assigned to Worcester County Jail (WCJ) in Snow Hill, Maryland (MD), on the date of his death. An autopsy ruled Mr. AKINYEMI's death a suicide.

Prior to ICE custody, on December 13, 2019, Mr. AKINYEMI presented to U.S. Customs and Border Protection for admission into the U.S. at New York City, New York, as a B2 non-immigrant with authorization to remain in the U.S. for a temporary period not to exceed June 12,

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2018; however, he remained beyond that date. On July 23, 2019, Enforcement and Removal Operations (ERO) Baltimore, MD, served an immigration detainer on Mr. AKINYEMI after his arrest for sexual abuse of a minor. On December 19, 2019, Baltimore, MD, convicted and sentenced Mr. AKINYEMI to eight years' incarceration. On December 20, 2019, Baltimore City Jail in Baltimore, MD, released Mr. AKINYEMI to ERO Baltimore pursuant to the immigration detainer. ERO transferred and housed Mr. AKINYEMI at WCJ.

On December 20, 2019, during his intake screening, Mr. AKINYEMI reported a medical history of hypertension [(HTN) high blood pressure], controlled with medication, and admitted he assaulted his daughter, verbalized his inability to cope and manage stress, but denied suicidal ideations. Additionally, he appeared visibly agitated showing restlessness while sitting, intermittent standing, jumping during telephone conversations, expressive hand gestures, tapping on the desk, pacing, and rubbing his head. The intake nurse scheduled Mr. AKINYEMI for a mental health evaluation on December 23, 2019, and WCJ custody housed him on a maximum-security unit pending classification and tuberculosis clearance.

On December 21, 2019, at approximately 5:03 a.m. a custody officer (CO) found Mr. AKINYEMI unresponsive, hanging from a wall vent with a ligature, and radioed for medical assistance. At approximately, 5:04 a.m., a nurse, shift supervisor, and another CO arrived, removed Mr. AKINYEMI from the hanging position, checked for responsiveness, started chest compressions, and requested EMS activation and the automated external defibrillator (AED). At approximately 5:05 a.m., the Snow Hill, MD, voluntary fire department and advanced life support emergency medical services (EMS) personnel arrived, assumed care, and approximately 20 minutes later pronounced Mr. AKINYEMI deceased.

Mortality Finding:

As a result of Mr. AKINYEMI's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) unanimously determined that Mr. AKINYEMI's medical care at WCJ was not provided within the safe limits of practice. Specifically, the MRC determined WCJ's care or lack thereof, could pose a significant future risk. Overall findings identified were: 1) the licensed practical nurse (LPN) did not use cervical spine precautions when removing Mr. AKINYEMI from the hanging position, 2) WCJ did not provide adequate staffing, 3) and Mr. AKINYEMI was not scheduled for a follow-up or timely referral to a mental health provider after he reported during his intake screening that he feared losing his family, expressed an inability to cope emotionally and manage stress.

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The MRC convened on February 19, 2020, and consisted of the following members: Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) MD, IHSC Eastern Regional Clinical Director, physician reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) Nurse Practitioner - certified, IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC), administration and advanced practice provider (APP) reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F) RN, Chief Nurse, nurse reviewer; Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) Nurse Practitioner - certified, APP reviewer; Dr. (b)(6),(b)(7)(C),(b)(7)(F) Chief - Tele-Psychiatrist, mental health reviewer.

The following summary of events was based on: 1) Mr. AKINYEMI's WCJ medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. AKINYEMI's WCJ detention file; 5) Mr. AKINYEMI's autopsy report; 6) video footage from December 20, 2019; 7) an on-site review and staff interviews conducted by fact-finder Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) and IHSC's Behavioral Health Unit Chief, CAPT (b)(6),(b)(7)(C),(b)(7)(F) at WCJ on January 14 - 16, 2020; and 8) applicable WCJ policies and procedures, and ICE Detention Standards.

Summary of Events

On December 20, 2019 at approximately 8:30 p.m., a WCJ CO completed Mr. AKINYEMI's booking and preliminary medical screening. Mr. AKINYEMI reported a medical history of HTN, controlled with medication. Mr. AKINYEMI did not exhibit any abnormal behavior during the preliminary screening process, but the CO described Mr. AKINYEMI's behavior as "agitated," during a telephone call with an unknown person. In review of the processing video footage, Mr. AKINYEMI was observed visibly agitated showing restlessness while sitting, intermittent standing and jumping during the telephone conversation, expressive hand gestures, and tapping on the desk. The CO verbally redirected Mr. AKINYEMI multiple times during his telephone conversation. After the telephone call, Mr. AKINYEMI began pacing, rubbing his head, and making expressive hand gestures. The CO did not report Mr. AKINYEMI's behavior to medical or custody leadership.

The detainees, who arrived and were held in the holding cell at WCJ with Mr. AKINYEMI, reported he appeared anxious, agitated, emotionally distressed, and would hit himself, while repeating, "why, why, why?" The detainees did not report Mr. AKINYEMI's behavior to medical or custody staff.

- At approximately 10:05 p.m., an LPN completed Mr. AKINYEMI's medical intake screening. Mr. AKINYEMI reported taking lisinopril and hydrochlorothiazide (anti-hypertensive medication) for his HTN. His vital signs were normal, except for a mildly elevated blood pressure (BP) – 140/90 mmHg (normal 90/60 – 120/80 mmHg). During the mental health and Prison Rape Elimination Act (PREA) screening, Mr. AKINYEMI denied a history of mental illness, suicidal, or homicidal ideations, but admitted to

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assaulting his daughter. Mr. AKINYEMI continued repeating, “He did something he should not have,” and verbalized concerns of losing his family, inability to cope emotionally, and managing his stress. The LPN recorded Mr. AKINYEMI’s mood and behavior as appropriate, scheduled a mental health evaluation for Mr. AKINYEMI on December 23, 2019, because he reported assaulting his daughter, and informed the registered nurse (RN) of Mr. AKINYEMI’s statements. However, the RN did not inform the on-call provider of Mr. AKINYEMI’s mental health status, when she called to request medication continuation. After the intake screening, WCJ custody housed Mr. AKINYEMI on a maximum-security unit pending classification and tuberculosis clearance.

On December 21, 2019, at approximately 12:03 a.m., 2:00 a.m., and 3:47 a.m., the night shift CO performed official counts, and Mr. AKINYEMI appeared asleep in his bed.

- At approximately 4:43 a.m., the CO conducted rounds in the maximum-security unit and Mr. AKINYEMI appeared asleep, covered with a blanket from head to toe.
- At approximately 5:03 a.m., the CO began to awaken detainees in preparation for medication administration and discovered Mr. AKINYEMI hanging from the wall vent with a ligature, consisting of five socks tied together. The CO radioed the shift sergeant (Sgt.), requested he report to his location with medical staff and the 911 tool (a sharp-like tool used to cut down detainee from a hanging position).
- At approximately 5:04 a.m., the Sgt., an LPN, and another CO responded with the 911 tool, cut the ligature from around Mr. AKINYEMI’s neck, placed him on his back, and checked for responsiveness. The LPN observed Mr. AKINYEMI as pulseless and not breathing. She then began chest compressions, requested EMS activation, and the AED. A CO retrieved the AED, placed it on Mr. AKINYEMI’s chest, but no shock was advised. [Investigator’s note: (b)(7)(E)]
(b)(7)(E)
(b)(7)(E)
- At approximately 5:05 a.m., the Snow Hill, MD voluntary fire department and advanced life support EMS personnel arrived on scene and assumed care. EMS personnel observed Mr. AKINYEMI as unresponsive, not breathing, pulseless, and his skin was cold to touch. The EMS personnel transferred Mr. AKINYEMI to their cardiac monitor, which showed an asystole rhythm (also known as flatline, a condition of cardiac arrest where there is no discernible electrical activity in the heart).

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- At approximately 5:23 a.m., the EMS personnel contacted the on-call physician at Peninsula Regional Medical Center in Salisbury, MD, and requested orders to terminate cardiopulmonary resuscitation efforts and pronounced Mr. AKINYEMI deceased.
- At approximately 5:50 a.m., an LPN notified the on-call provider of Mr. AKINYEMI's death.

The final autopsy report revealed Mr. AKINYEMI's cause of death was hanging. The coroner noted Mr. AKINYEMI's injuries were consistent with a self-inflicted act. The physical markings observed on one side of the thyroid cartilage was also consistent with the ligature that accompanied Mr. AKINYEMI's body. The coroner determined the manner of death as suicide.

CAPT (b)(6),(b)(7)(C),(b)(7)(F) Behavioral Health Unit Chief, performed Mr. AKINYEMI's psychological autopsy (procedure for analyzing the cause of death by reconstructing what the person thought, felt, and did before death, based on gathered information). The final psychological autopsy report is pending.

ICE detention standards used for this review: ICE National Detention Standards (NDS) 2000. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) custody and medical staff's response time was within one minute of notification; 2) the LPN completed Mr. AKINYEMI's intake screening within six hours of arrival; and, 3) although WCJ is not currently accredited by the National Commission on Correctional Health Care (NCCHC) or the American Correctional Association (ACA), WCJ health care services have implemented local policies to coincide with these standards.

Weaknesses

1. Staffing.

- (b)(7)(E)
- WCJ staffing model was inadequate to address the medical needs of the assigned population.

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Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.C. *Medical Personnel*.

2. Patient safety.

- On December 21, 2019, during the medical emergency, the LPN failed to use cervical spine precautions to immobilize Mr. AKINYEMI's head and avoid further injury.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- Up to Date, *Evaluation and initial management of cervical spinal column injuries in adults (2020)*. Accessed March 5, 2020. https://www.uptodate.com/contents/evaluation-and-initial-management-of-cervical-spinal-column-injuries-in-adults?source=history_widget.

3. Mental health screening.

- On December 20, 2019, during the medical intake screening, Mr. AKINYEMI verbalized concerns of losing his family, inability to cope emotionally, and managing stress; however, the RN did not timely refer Mr. AKINYEMI to a mental health provider, notify the on-call provider, or initiate mental health observation protocol.
- The WCJ booking officer did not report Mr. AKINYEMI's agitated behavior to medical or custody leadership.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.D. *Medical Screening*.
- WCJ, Standard 02, Inmate Safety Policy, *Emergency Medical Services*.
- Wellpath's Worcester County, MD Detention Center Policies and Procedures: HCD-100_F-03, *Mental Health Services*.

4. Training.

- Several WCJ staff reported, no participation in a man-down drill, scenario-based patient, or situational simulation exercises.
- Several WCJ staff reported suicide prevention and intervention training inconsistencies regarding frequency, content, and method.

Applicable standards of care for this finding:

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- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- WCJ, Standard 02, Inmate Safety Policy, *Emergency Medical Services*.
- Wellpath's Worcester County, MD Detention Center Policies and Procedures: HCD-100_F-03, *Mental Health Services*.

5. First aid and cardiopulmonary resuscitation.

- During the emergency response, the LPN did not have access to medical equipment; specifically, the bag valve mask and was unable to provide rescue breathing.
- Correctional staff were not familiar with the location of the AED.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- WCJ, Standard 02, Inmate Safety Policy, *Emergency Medical Services*.
- American Heart Association, Highlights of the 2015 American Heart Association Guidelines Update for CPR and ECC (Emergency Cardiovascular Care). Accessed March 5, 2020. <https://eccguidelines.heart.org/wp-content/uploads/2015/10/2015-AHA-Guidelines-Highlights-English.pdf>.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, WCJ's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.